

Plaintiff contends, without supporting evidence, that she was not provided with a copy of the AAA Arbitration Rules which supports a finding of unconscionability, relying on the case *Harper v. Ultimo* (2003) 113 Cal.App.4th 1402. The Court in *Baltazar* explained decisions such as *Harper v. Ultimo*, stating with regard to the failure to provide the arbitration rules in that case that "the plaintiff's procedural unconscionability claim depended in some manner on the arbitration rules in question. [Citations omitted.] These cases thus stand for the proposition that courts will more closely scrutinize the substantive unconscionability of terms that were 'artfully hidden' by the simple expedient of incorporating them by reference rather than including them in or attaching them to the arbitration agreement. [Citation omitted.]" (*Baltazar, supra*, 62 Cal.4th at 1246 [emphasis added].)

Plaintiff has not cited any provision of the Arbitration Agreement itself or the AAA Arbitration Rules that is overly harsh, one-sided, contrary to public policy, or otherwise unfair. In contrast, in *Harper v. Ultimo, supra*, the Better Business Bureau arbitration rules not provided to the customer included provisions severely and unexpectedly restricting the remedies available to the complaining party. (*Harper v. Ultimo, supra*, 113 Cal.App.4th at 1405-1406 [construction contract damage cap of \$2,500 on top of limitation of remedies against contractor under construction project to refund, completion of work, repair costs, or out-of-pocket property damage substantively unconscionable].)

Plaintiff has not shown any substantive unconscionability, which is essential to the unconscionability defense, as both procedural and substantive unconscionability must be proven to render the Arbitration Agreement unenforceable. The motion is therefore **granted**.

16. 9:00 AM CASE NUMBER: MSC21-01948
CASE NAME: KATZ VS. ESKATON PROPERTIES
HEARING ON SUMMARY MOTION
FILED BY: ESKATON PROPERTIES, INC. A CALIFORNIA CORPORATION
TENTATIVE RULING:

Before the Court is Defendants Eskaton Properties, Inc. and The Reutlinger Community's Motion (collectively "Defendants") for Summary Judgment or, in the alternative, Summary Adjudication ("MSA").

Procedural History and Factual Background

Plaintiffs Alexander Katz and Julia Mirman are the adult children of Marat Katz and the successors in interest and heirs of Marat Katz. Given the same last name of Alexander and Marat Katz, the Court will refer to each by their first name to avoid confusion. The Court means no disrespect in using this procedure. The Court notes Defendants' expert, Morgan Katz, M.D., also has the surname Katz, so the use of first names will hopefully decrease any confusion.

Marat was admitted to The Traditions memory care unit of The Reutlinger on March 27, 2020. (UMF 1). His admission diagnosis included mild cognitive impairment, unspecified dementia with behavioral disturbance, and unspecified focal traumatic brain injury. (UMF 4.) On August 19, 2020, during this stay at The Reutlinger, he was briefly hospitalized at San Ramon Regional Medical Center. (UMF 5.) He then returned to Traditions. (*Ibid.*)

On August 22, after Marat returned from San Ramon, The Reutlinger tested him for COVID-19. (UMF 21.) The test came back negative on August 26, 2020. (*Ibid.*) Later that same day, Marat was found unresponsive in his room and emergently transferred back to the hospital. (UMF 22.) After being re-admitted to the hospital, he did not receive COVID-directed therapy until August 31. (UMF 23.)

The Reutlinger collected multiple COVID tests from Marat around this time. (UMF 25.) He tested negative in July and on August 22. (UMF 25 – Allen Decl. ¶15 c.) A test collected on August 23 returned positive. (*Ibid.*) Subsequent testing at John Muir returned negative results. (*Ibid.*)

Marat was discharged from Traditions on August 22, 2020. (UMF 6.) He died on September 15, 2020, at John Muir Medical Center. (UMF 7.) His death certificate indicates that the immediate cause of death was “hypoxic respiratory failure.” (UMF 8.) The underlying cause is identified as COVID-19. (*Ibid.*)

The original complaint was filed on September 16, 2021. The operative First Amended Complaint was filed on May 11, 2022. It alleges three causes of action: (1) reckless or wilful neglect of an Elderly Adult; (2) wrongful death; and (3) negligence.

Initial Considerations

“If evidence in support of or in opposition to a motion exceeds 25 pages, the evidence must be separately bound and **must include a table of contents.**” (Cal. R. Ct. 3.1350(g) emphasis added.) Plaintiffs filed a 20-part collection of exhibits in support of their opposition with 68 exhibits and over 8,000 pages. No table of contents was provided, making the Court’s job exponentially more difficult in locating the proper exhibit when cited.

When a deposition is used as an exhibit, “the exhibit must contain only the relevant pages of the transcript.” (Cal. R. Ct. 3.1116 (b).) In addition, the “relevant portion of any testimony in the deposition **must be marked** in a manner that calls attention to the testimony.” (Cal. R. Ct. 3.1116 (c) emphasis added.) Normally, this is done by either highlighting or bracketing the lines being cited in the Memorandum and/or Separate Statement.

Among the 68 exhibits are the deposition transcripts of ten separate individuals. The full transcript of each deposition – including the word indexes – are attached. None of the transcripts includes markings of any relevant portions. A review of Plaintiffs’ Opposition and Response to the Separate Statement shows that Plaintiffs only cite three of these depositions: Anna Catalan, Arlisha Tillman, and Nanci Bustamante. The inclusion of these voluminous exhibits, a minority of which are actually cited, without citations or highlights to relevant portions is unnecessary and only creates an additional burden on the Court.

Additionally, all documents filed electronically “must be text searchable when technologically feasible...” (Cal. R. Ct. 2.256 (b)(3).) Only some of the deposition transcripts were electronically searchable, making the Court’s work incredibly more time consuming.

Standard

Summary judgment is proper if all the papers submitted show that there is no triable issue as to any material fact and that the moving party is entitled to a judgment as a matter of law. (Code Civ. Proc. §

437c(c).) The “moving papers must demonstrate that ‘material facts’ are undisputed and that the movant is entitled to judgment as a matter of law. (*Id.* at (b)(1), (c).) “The moving party’s papers are strictly construed whereas those of the opposing party are liberally construed, with all doubts about the granting of the motion resolved in favor of the opposing party.” (*Y.K.A. Industries, Inc. v. Redevelopment Agency of City of San Jose* (2009) 174 Cal.App.4th 339, 352.)

The moving party “bears an initial burden of production to make a prima facie showing of the nonexistence of any triable issue of material fact; *if* he carries his burden of production, he causes a shift, and the opposing party is then subjected to a burden of production of his own to make a prima facie showing of the existence of a triable issue of material fact.” (*Id.* at 353 quoting *Aguilar v. Atlantic Richfield Co.* (2001) 25 Cal.4th 826, 850, italics in original.) Thus, a defendant moving for summary judgment “must make a prima facie showing, i.e., ‘he must present evidence that would require a reasonable trier of fact *not* to find any underlying material fact more likely than not—otherwise, *he* would not be entitled to judgment *as a matter of law*, but would have to present *his* evidence to a trier of fact.” (*Ibid.* quoting *Aguilar*, *supra*, 174 Cal.App.4th at 851.)

Analysis

Negligence

While the third cause of action is generally labeled one for ‘negligence,’ Defendants contend it is a claim for ‘professional negligence.’

“Professional negligence” applies to “a negligent act or omission to act by a health care provider *in the rendering of professional services*, which act or omission is the proximate cause of a personal injury or wrongful death, provided that such services are within the scope of services for which the provider is licensed and which are not within any restriction imposed by the licensing agency or licensed hospital.” (*So v. Shin* (2013) 212 Cal.App.4th 652, 663.) “Courts have broadly interpreted ‘in the rendering of professional services,’ concluding that a negligent act that occurs in the rendering of services for which the health care provider is licensed is professional negligence.” (*Ibid.*)

Plaintiffs do not dispute this categorization in their Opposition. The claims in this case properly fit within the above definition and will be addressed as a professional negligence claim.

“The elements of a cause of action for professional negligence are failure to use the skill and care that a reasonably careful professional operating in the field would have used in similar circumstances, which failure proximately causes damage to plaintiff.” (*Cyr v. McGovran* (2012) 206 Cal.App.4th 645, 651.) The elements of duty, breach, and causation in professional negligence cases required expert opinion testimony. (See e.g. *Scott v. Rayhrer* (2010) 185 Cal.App.4th 1535, 1542; *Bushling v. Fremont Medical Center* (2004) 117 Cal.App.4th 493, 509.)

The general rule is that causation is a question of fact not resolvable on summary judgment; “[t]he issue of causation may be decided as a question of law only if, under undisputed facts, there is no room for a reasonable difference of opinion.” (*Nichols v. Keller* (1993) 15 Cal.App.4th 1672, 1687.)

Defendants submit the declaration of Josh Allen, RN, Certified Assisted Living Nurse (C-AL). He has approximately 28 years of experience in the operations of assisted living communities. Specifically, with relation to COVID-19, he was retained by the California Department of Social Services (CDSS) as

an expert consultant on COVID-19 preparedness and response in assisted living communities from 2019 to 2024.

Mr. Allen notes that Marat was admitted to Traditions on March 27, 2020 – eight days after California issued its statewide shelter-in-place order. He explains that The Reutlinger developed “comprehensive, CDC-sourced COVID-19 mitigation policies that were consistent with the industry standard of care during the relevant period.” (¶ 13.) These protocols included universal mandatory masking for all direct care staff. (¶ 13 a.) Mr. Allen confirms that despite these protocols, a “CDSS Community Care Licensing citation confirms, that on August 21, 2020, Mr. Katz’s caregivers were observed not wearing required PPE while providing care to him.” (¶ 15.) This was a “violation of [The Reutlinger’s] own written policies and of the applicable standard of care.” (*Ibid.*)

Mr. Allen contends that this citation reflects a “single-date observation,” and does not establish that “PPE non-compliance was a widespread, systematic, or longstanding practice” at The Reutlinger during the relevant time. (¶ 15 a.) Mr. Allen does not explain or opine as to the actual day-to-day operations at The Reutlinger but merely discusses their protocols and the fact that a single citation was received.

Defendants also present the declaration of Morgan Katz, M.D., who is an infectious disease physician and Associate Professor of Infectious Disease at John Hopkins University. He has extensive experience working with COVID protocols and procedures, specifically at nursing homes. He outlines the series of events and COVID testing for Marat during his stay at The Reutlinger.

- July 17: COVID test due to a positive case in the facility
 - Negative results received July 24
- August 19 – fall, taken by ambulance to San Ramon Medical. At Medical Center for about 5 hours.
- August 22 – COVID test at 9 a.m. due to positive case in the facility reported August 21.
 - Negative results received August 26
- August 22 – about 6 p.m. found unresponsive in room – taken to San Ramon Medical.
- August 23 – about 4 p.m. another positive case of COVID at Reutlinger reported.
- August 23 – Marat transferred to John Muir – tested for COVID
 - Positive result
- August 23 – September 15: Marat has continued problems and continues to receive treatment at John Muir
- September 15 – family elects to proceed with comfort care due to his poor prognosis. Marat dies that day.

Dr. Katz opines that the care provided by Reutlinger satisfied the applicable standard of care. (¶ 29.) He indicates that he reviewed all the policies and procedures and opines that there “is no evidence of

a breach in infection prevention policy or practice at Reutlinger Communities that led to Mr. Katz's outcome." (¶ 30.) He does not explain, however, how he came to the conclusion that there is "no evidence of a breach" based solely on a review of the policies and protocols. He does not indicate that he inspected the facility during the relevant time, nor that he was made aware of the actual day-to-day **implementation** of the policies and procedures.

Dr. Katz indicates that it is "impossible to determine the definitive cause of [Marat's] exposure to the SARS-CoV-2 virus, particularly given the fact that he was living in an open memory care unit where masking and confining residents to their room is very challenging." (¶ 31.) This statement appears to undermine his conclusion that there is 'no evidence' of a breach of the protocols. Given the 'challenging' conditions and other evidence discussed below, it would appear that there were breaches of the protocols. Notably, Dr. Katz's declaration indicates that he did not review the pictures submitted by Ms. Mirman or the deposition of Ms. Catalan which include the descriptions and pictures of the lack of use of PPE at the facility.

The declaration of Jay Luxenberg, M.D., also presented by Defendants, indicates that he believed that The Reutlinger implemented proper protocols during the relevant time. (¶ 19.) Without discussing what actually occurred there, he opines that "isolated lapses in PPE compliance does not establish causations or negligence," in that they were common across healthcare settings. (¶ 20.) Notably, he does not opine that Marat did not acquire COVID while at The Reutlinger. Instead, he states that there is no reliable medical basis to conclude that "any alleged PPE deviation caused [Marat's] infection, as opposed to the well-known background risk in all similar facilities at that time." (¶ 22.)

As with Dr. Katz's declaration, there is no statement that Marat did not (or could not have) acquired COVID while at The Reutlinger. Instead, his opinion discusses protocols and procedures and 'isolated lapses.' There is no discussion as to whether the day-to-day procedures at The Reutlinger included only 'isolated lapses' or if there were continuing and pervasive lapses. Nor is there a discussion as to at what point the 'lapses' are no longer 'isolated.'

Plaintiffs submit the declaration of Peter Chin-Hong, M.D. He is board certified in Internal Medicine and Infectious Disease and has practiced clinical infectious diseases for over twenty years. His review of the records led him to his explanation as to how Marat acquired the COVID virus. He notes that a caregiver at The Reutlinger ("LA") worked in the Traditions unit that housed Marat on August 7-8, 11-14 and 17, 2020. (¶ 27.) Her shifts were 8.5 hours with a break. On August 18, she reported a fever and was given a COVID-19 test. The test came back positive on August 21.

These dates in early and mid-August coincide with when Marat was diagnosed with COVID and began having COVID related symptoms. Following Marat's admission to the hospital, The Reutlinger continued to have employees and residents test positive for COVID. A housekeeper tested positive on August 24. (¶ 33.) Two residents were tested on August 22, with positive results coming back on August 27. (*Ibid.*)

Dr. Chin-Hong notes that the materials he reviewed "describe multiple concerns suggesting that implementation did not consistently match the facility's written policies, which required all employees and visitors to wear surgical masks at all times and for residents to wear masks while in the presence of caregivers. (¶ 36.) "From an infectious disease perspective, these observations are

medically significant because outbreaks commonly arise not from the absence of written policies but from repeated failures in implementation. Multiple relatively small deviations from accepted infection-prevention practices may combine to produce substantial increases in transmission risk.” (¶ 37.)

“Here, the temporal relationship between infected staff and residents, the clustering of infections within a relatively small residential unit, documented deficiencies in infection-control implementation, and the absence of a more persuasive alternative exposure together support the conclusion that Mr. Katz more likely than not acquired SARS-CoV-2 within the Traditions memory care unit.” (Chin-Hong Decl. ¶ 26.)

“While any isolated lapse might not necessarily result in transmission, repeated deficiencies occurring during an active outbreak increase the probability of healthcare worker-to-resident transmission. This is especially important in a memory care unit where prolonged close contact between caregivers and residents is unavoidable and residents cannot reliably participate in their own infection prevention by masking.” (¶ 45.)

He also found it significant that the housing unit was relatively small – housing approximately nineteen residents. (¶ 55.) However, during the month of August several cases of COVID were documented within the unit. A Traditions caregiver (“LA”) tested positive, as did a Traditions resident (“CG”) and an Assisted Living resident (“PC”), and a Reutlinger housekeeper who worked in the Traditions unit. (*Ibid.*) He notes that the “temporal clustering of infections among residents and staff within the same facility is epidemiologically significant.” (*Ibid.*)

Taken together, all the evidence led Dr. Chin-Hong to determine that Marat “more likely than not acquired SARS-CoV-2 within the Traditions unit.” (¶ 58.)

Dr. Chin-Hong also addressed Defendants’ theory that Marat could have acquired COVID while at the hospital. He notes that Marat was at San Ramon on August 19 for about four and a half hours – which is a relatively small window. Especially, compared to the full-time living situation at The Reutlinger where there were known COVID positive people. This was also during the time when complaints about lack of PPE were occurring.

He also addresses Defendants’ expert’s positions. While he generally agrees with “several general principles contained in those reports,” he differs in the “application of those principles to the facts of this case.” (¶ 72-73.) He notes that the defense experts (as noted above) place “substantial emphasis on the existence of written infection control policies.” (¶ 74.) Dr. Chin-Hong notes, however, that “effective infection prevention is determined by implementation rather than documentation,” and there is sufficient evidence to establish that the policies and procedures were not properly implemented and/or enforced.

Plaintiffs also submit the declaration of David J. Farrell, a Licensed Nursing Home Administrator (LNHA) with over 34 years of experience. His declaration reiterates the series of events outlined above regarding the other COVID cases in the unit and how that credibly led to Marat’s acquiring the COVID virus. (¶¶ 11-27.) He also directly addresses the opinions of Mr. Allen outlined above. While Mr. Farrell acknowledges there were challenges regarding the availability of PPE and the like, it was not as dire as Mr. Allen appears to indicate. (¶ 28.) He also agrees that The Reutlinger developed

policies and procedures that “were aligned with the infection control standards at that time.” (§ 29.) He opines, however, that Mr. Allen “fails to acknowledge that the defendants did not follow their own COVID-19 mitigation policies and procedures....” (*Ibid.*)

Mr. Farrell also disagrees with Mr. Allen and Dr. Katz’s opinions that the ‘breaches’ in the protocols were not systematic and did not lead to Marat’s infection. He notes that neither of Defendants’ experts address the evidence related to those breaches but instead generalize without review of the specific facts in this case. (§ 30-31.)

The evidence includes a note signed by seven The Reutlinger employees which appears to confirm that there were insufficient masks for all the workers. (P’s Ex. 66.) It states in part: “We want enough of N95 masks. Yesterday we had to borrow from Skilled because we didn’t have any. ... It is not acceptable.” (*Ibid.*) Ms. Catalan, an employee during the relevant time, also testified that Personal Protective Equipment (PPE) was not timely refilled and they often did not have enough masks and the like. (Catalan Depo. at 28:5-25.)

Julia Mirman also submits a declaration wherein she indicates that she would Facetime with Marat while he was at The Reutlinger. She submits several photo screenshots from those interactions which show caregivers within close proximity of Marat improperly wearing face masks (i.e. not covering their nose and mouth) or not wearing a mask at all. Below is just a sample:



It is worth nothing that according to Mr. Allen’s and Dr. Katz’s declarations, neither of them reviewed the pictures submitted by Ms. Mirman or the deposition of Ms. Catalan which include the descriptions and pictures of the lack of use of PPE at the facility.

Summary and Conclusion Regarding Negligence Claim

The general rule is that causation is a question of fact not resolvable on summary judgment; “[t]he issue of causation may be decided as a question of law only if, under undisputed facts, there is no

room for a reasonable difference of opinion.” (*Nichols v. Keller* (1993) 15 Cal.App.4th 1672, 1687.)

Defendants argue that there is no evidence that their negligence caused Marat’s COVID-19 infection. Their experts explain that, given the uniqueness of the situation and the ever-changing landscape of new information, The Reutlinger implemented policies and procedures that were aligned with the infection control standards at that time and in accordance with CDC guidance. Plaintiffs do not necessarily disagree with that position. They do, however, argue that the policies and procedures were not effectively implemented – and the failure to properly follow the policies and procedures is what led to Marat’s acquiring the COVID-19 virus.

Plaintiffs submit evidence showing that the staff were concerned about the lack of PPE. There is also photographic evidence of caregivers not wearing, or improperly wearing, masks while in direct contact with Marat. There is also evidence that several residents and caregivers tested positive for COVID during the same timeframe as Marat. All of this could lead a trier of fact to determine that Defendants’ failure to properly execute its policies and procedures led to Marat’s illness. Marat’s COVID had to come from somewhere. Given that he was confined to The Reutlinger during the relevant time, persons at The Reutlinger tested positive for COVID during the same timeframe, and there is evidence that the caregivers were not properly wearing masks during that time, plaintiffs present a credible theory as to how he contracted the virus.

Defendants attempt to argue that Marat could have acquired the illness on his trips to the hospital. As noted above, the first trip lasted less than five hours. This is an incredibly small fraction of the time at issue – being the mid to late weeks in August. There is also no evidence that Marat was ever exposed to the COVID-19 virus during this trip. Defendants present no evidence of any other positive tests for people at the hospital, nor any evidence suggesting that the hospital personnel were not properly using PPE. In other words, there is no evidence showing how Marat could have acquired the virus while at the hospital.

“‘Speculation ... is not evidence’ that can be utilized in opposing a motion for summary judgment.” [citation] Speculation also differs from a reasonable inference. ‘When opposition to a motion for summary judgment is based on inferences, those inferences must be reasonably deducible from the evidence, and not such as are derived from speculation, conjecture, imagination, or guesswork.’” (*Advent, Inc. v. National Union Fire Ins. Co. of Pittsburgh, PA* (2016) 6 Cal.App.5th 443, 459.)

Based on the above, Defendants’ motion for summary adjudication of the negligence cause of action is **denied**.

Plaintiffs also appear to assert a theory of liability related to Defendants prescribing of antipsychotic drugs. They allege that Marat was previously prescribed these drugs and had a very adverse reaction. As such, Defendants were informed not to give them to Marat. Despite these warnings, Defendants continued to prescribe and administer the drugs which caused and/or contributed to Marat’s falls. The injury contributed to Marat’s susceptibility the COVID virus and/or his cause of death.

The Court need not address these alternative arguments at this time given the Court’s findings discussed above that support Plaintiff’s negligence cause of action.

Wrongful Death

“The elements of a cause of action for wrongful death are the tort (negligence or wrongful act), the resulting death, and the damages, consisting of the *pecuniary loss* suffered by the *heirs*.” (*Lattimore v. Dickey* (2015) 239 Cal.App.4th 959, 968, *italics* in original.)

Defendants’ position regarding this cause of action is that because Plaintiffs have failed to establish the other causes of action – in particular the negligence cause of action – this claim also fails. (Motion at Sec. 8.) As stated above, the Court denied Defendants’ request for summary adjudication as to the negligence cause of action. As such, Plaintiff’s have properly asserted the tort, i.e. negligence. Marat’s acquiring of COVID led to his death according to the Death Certificate as well as Defendant’s own expert. As Dr. Luxenberg opine: “Mr. Katz’s death was caused by **hypoxic respiratory failure due to COVID-19....**” (¶ 11e emphasis in original.)

Accordingly, Defendants’ request for summary adjudication of the wrongful death cause of action is also **denied**.

Elder Abuse – Neglect

The Elder Abuse and Dependent Adult Civil Protection Act (the “Act”) provides in part that where it is proven “by clear and convincing evidence that a defendant is liable for physical abuse ..., [or] neglect ... [of an elderly or dependent adult], and that the defendant has been guilty of recklessness, oppression, fraud, or malice in the commission of the abuse,” the plaintiff is entitled to enhanced damages. (Wel & Inst. Code §15657.) Plaintiffs claim is based on allegations of ‘neglect’ of Marat.

The elements of a cause of action for Elder Abuse based on neglect are: (1) defendant had a substantial caretaking or custodial relationship with plaintiff, involving ongoing responsibility for plaintiff’s basic needs, which an able-bodied and fully competent adult would ordinarily be capable of managing without assistance; (2) the plaintiff was 65 years of age or older while in defendant’s care or custody, (3) that defendant failed to use the degree of care that a reasonable person in the same situation would have used in providing for plaintiff’s basic needs, (4) that plaintiff was harmed, and (5) defendant’s conduct was a substantial factor in causing plaintiff’s harm. (See Welf. & Inst. Code §§ 15610 *et seq.*; *Covenant Care, Inc. v. Superior Court* (2004) 32 Cal.4th 771, 783; CACI 3103.)

Defendants focus their argument on elements 3 and 5 – arguing that the actions alleged against them do not constitute ‘neglect’ under the statute and there is no evidence that their actions were a substantial factor in causing Marat’s injuries and death.

“Neglect includes, but is not limited to, all of the following: ... (2) Failure to provide medical care for physical and mental health needs. ... (3) Failure to protect from health and safety hazards....” (Cal. Welf. & Inst. Code §15610.57(b).)

“[T]he acts proscribed by [the Act] do not include acts of simple professional negligence, but refer to forms of abuse or neglect performed with some state of culpability greater than mere negligence.” (*Delaney v. Baker* (1999) 20 Cal.4th 23, 32.) In cases where the party argues that proper care was withheld (as is the case here), “the trier of fact must determine whether there is a significant pattern of withholding portions or types of care.” (*Sababin v. Superior Court* (2006) 144 Cal.App.4th 81, 90.) “A significant pattern is one that involves repeated withholding of care and leads to the conclusion

that the pattern was the result of choice or deliberate indifference.” (*Ibid.*)

Here, the alleged ‘withholding of care’ pertains to the failure of the caregivers to don proper PPE including masks when caring for Marat. The evidence discussed above indicates that while The Reutlinger may have enacted policies and procedures that meet the standards for the time there was a failure to adequately implement them and ensure that the staff were actually abiding by the policies. It is for the trier of fact to determine if the failures to wear proper PPE were repeated and pervasive enough to establish that the actions were the result of more than mere negligence.

Enhanced Remedies

Plaintiffs seek enhanced remedies under the Elder Abuse Act. To recover the enhanced remedies, the “plaintiff must prove ‘by clear and convincing evidence’ that ‘the defendant has been guilty of recklessness, oppression, fraud, or malice in the commission of’ the neglect.” (*Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, 405.) In a case such as this where the enhanced remedies are sought against a corporate employer, the “standards set forth in subdivision (b) of Section 3294 of the Civil Code regarding the imposition of punitive damages on an employer based upon the acts of an employee shall be satisfied...” (Wel. & Inst. Code § 15657 (c).)

An award of punitive damages against a corporation “must rest on the malice of the corporation’s employees.” (*Cruz v. Homebase* (2000) 83 Cal.App.4th 160, 167.) The “law does not impute every employee’s malice to the corporation.” (*Cruz*, supra, 83 Cal.App.4th at 167.) “Instead, the punitive damage statute requires proof of malice among corporate leaders: the ‘officer[s], director[s], or managing agent[s].’” (*Ibid.* quoting Cal. Code Civ. Proc. § 3294, subd. (d).)

A ‘managing agent’ is an employee “who ‘exercise[s] substantial discretionary authority over decisions that ultimately determine *corporate policy*.’” (*Ibid.* italics in original.) ‘Corporate policy’ is “the general principles which guide a corporation, or rules intended to be followed consistently over time in corporate operations.” (*Ibid.*) “A ‘managing agent’ is one with substantial authority over decisions that set these general principles and rules.” (*Id.* at 167-68.)

Defendants note the lack of any evidence identifying any particular officer, director, or managing agent that acted with recklessness, oppression, fraud, or malice. Plaintiffs do not even address this argument in their Opposition.

The only reference in Plaintiffs’ Opposition to a person in such a role is the following: “In August 2020, Arlisha Tillman, the SEIU representative for care givers at Reutlinger, asked to have a meeting with Jay Zimmer, the CEO of Reutlinger, to discuss employee complaints regarding management’s handling of Covid 19.” (Opp. at 9:2-4.) There is no citation to any evidence to support this statement.

“Matters set forth in points and authorities are not evidence.” (*Alki Partners, LP v. DB Fund Services, LLC* (2016) 4 Cal.App.5th 574, 590.) “Evidence appears elsewhere – in deposition testimony, discovery responses, and declarations.” (*Ibid.*) Even if there were evidence of this letter, it does not show that Mr. Zimmer was aware of the particular issues, that he had any direct oversight of those issues, or that he was involved in the implementation thereof.

Plaintiffs fail to point to any evidence to support their claim for the enhanced remedies, including attorneys’ fees and costs. Accordingly, Defendants’ motion for summary adjudication of this issue is

granted.

Deficient Discovery Responses

Defendants argue that they are entitled to summary judgment/adjudication based on Plaintiffs' deficient discovery responses, citing *Union Bank v. Superior Court* (1995) 31 Cal.App.4th 573. Specifically, it notes that Plaintiffs' responses related to requests seeking information pertinent to causation were never properly responded to. Instead, there were just blanket objections and statements that the information was subject to expert disclosures.

As stated in *Union Bank*:

“[A] moving defendant may rely on factually devoid discovery responses to shift the burden of proof pursuant to section 437c, subdivision (o)(2). Once the burden shifts as a result of the factually devoid discovery responses, the plaintiff must set forth the specific facts which prove the existence of a triable issue of material fact.”

Plaintiffs' factually devoid discovery responses could provide Defendants with the ability to shift the burden to Plaintiff to present facts which show there is a triable issue of material fact. As outlined above, the Court found that Plaintiffs have met that burden. As such, Defendants' argument that they are entitled to summary judgment due to Plaintiffs' deficient discovery responses is without merit.

Evidentiary Objections

The Court need only rule on those objections to evidence that were material to the disposition of the MSJ/MSA. (See Code Civ. Proc. § 437c(q).)

Plaintiffs' Objections

Plaintiffs' objections are all based on the assertion that portions Defendants' expert declarations are based on inadmissible evidence because (at least in part) the medical records which were reviewed and relied upon by those experts were not submitted to the Court. As such, the expert declarations are based on inadmissible hearsay and should properly be disregarded by the Court. Plaintiffs' cite *Garibay v. Hemmat* (2008) 161 Cal.App.4th 735 as support for their position.

Garibay involved a medical malpractice claim related to a tubal ligation procedure that was allegedly improperly performed. Defendants moved for summary judgment based exclusively on the declaration of their medical expert who reviewed Plaintiffs' medical records and opined that the defendant doctor complied with the applicable standard of care. Plaintiffs objected to the declaration based on hearsay, lack of foundation, and lack of personal knowledge – among other grounds. They asserted that the declaration was improper expert testimony because it was based on the records which were not properly before the court. Notably, the Plaintiff did not file a substantive opposition or submit any of its own evidence in support of its opposition. The trial judge sustained two of the objections but overruled the rest – allowing the declaration and granting summary judgment.

The Court of Appeal reversed noting that although hospital and medical records are hearsay, they can be admitted under the business records exception if they are properly authenticated. Without those records being authenticated and before the Court, the expert's declaration had no evidentiary basis.

The court found that the expert had “no personal knowledge of the underlying facts of the case, and attempted to testify to facts derived from medical and hospital records which were not properly before the court. Therefore, his declaration of alleged facts had no evidentiary foundation.”

Here, as noted by Defendants on Reply, the records are before the Court. Plaintiffs submitted the records of the Reutlinger Community, as well as Marat’s records from John Muir, San Ramon Regional Medical Center, and San Ramon Valley Fire Protection, among others. Plaintiffs’ own experts indicate they reviewed and relied upon these same documents. As such, these documents are ‘properly before the court,’ unlike in *Garibay* where they were submitted to the court at all.

Plaintiffs Objections 1-9: Overruled.

Defendant’s Objections

“Each written objection ***must be numbered*** consecutively” (Cal. R. Ct. 3.1354(b).) Defendant fails to include this basic information.

Defendants’ Objections 1-24: Overruled.

Conclusion

Defendants’ motion for summary judgment is **denied**. Defendants’ motion for summary adjudication is:

- **Denied** as to the negligence cause of action.
- **Denied** as to the wrongful death cause of action.
- **Denied** as to the elder abuse – neglect cause of action.

Granted as to the request for enhanced remedies related to the elder abuse – neglect cause of action.

17. 9:00 AM CASE NUMBER: N26-0572

CASE NAME: PETITION OF: JOSE GRACIANO HUEZO

***HEARING ON MOTION IN RE: PETITION**

FILED BY:

TENTATIVE RULING:

Introduction

Petitioner Jose Graciano Huezo (“Petitioner”) filed the instant petition seeking relief from the claim presentation requirements for a cause of action that accrued when his car was impounded by the Richmond Police Department in February 2023. On April 3, 2026, Petitioner filed the instant petition seeking relief from this Court from the claim presentation requirements.

For the reasons discussed below, the Court **denies the Petition for Relief from Claim Presentation Requirement.**

Factual Background